Trastuzumab emtansine (T-DM1) · metastatic

T-DM1 3.6 mg/kg IV every 21 days, adjusted if required, until progression or unacceptable toxicity. This metastatic course has no fixed 14-cycle endpoint; the postoperative early-breast regimen is a different card.

Source reconciled; independent clinical, pharmacy and patient review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Breathing

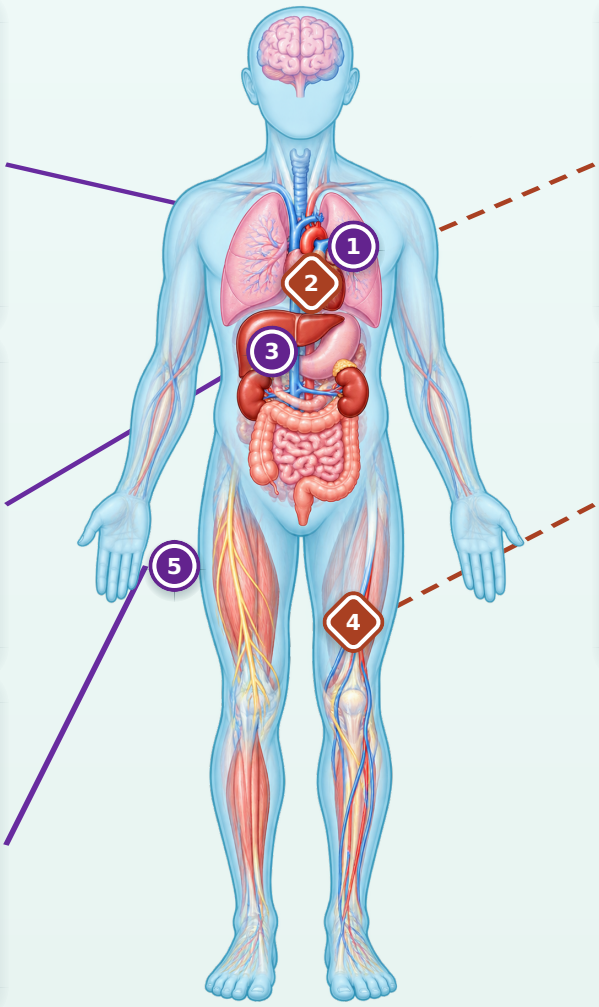
Trastuzumab emtansine (T-DM1)

New cough or breathlessness: seek prompt assessment.

2 Heart

Trastuzumab emtansine (T-DM1)

Keep echo checks. Report swelling, chest symptoms, fainting or rapid weight gain.



3 Liver

T-DM1

Keep liver blood tests. Report yellow eyes / skin, dark urine or significant abdominal symptoms.

4 Platelets / bleeding

T-DM1

Report unexplained bruising or bleeding; heavy or persistent bleeding needs urgent help.

5 Nerves

T-DM1

Report numbness or tingling, especially if it affects walking or hand use.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

Your treatment, at home.

Important medicine-specific effects

Trastuzumab emtansine (T-DM1)

- Low platelets and bleeding
- Liver injury, rarely serious
- Numbness or tingling
- Heart pumping-function reduction
- Rare serious lung inflammation
- Fatigue, nausea and infusion reactions

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Support for this medicine

T-DM1 has low emetogenic risk; review the low-risk antiemetic draft and individual contraindications.

Blood and heart appointments

FBC, renal and liver profile before every cycle. Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles.

Local treatment and endocrine therapy

Surgery and radiotherapy are coordinated by the MDT. When endocrine therapy is indicated, it has its own schedule and bone-health plan.

Pregnancy / contraception

Discuss pregnancy, fertility and contraception before treatment. Report pregnancy promptly; do not breastfeed during treatment or for 7 months afterwards. The team confirms the medicine-specific contraception duration.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

CUH · MON–FRI 8AM–5PM

087 181 9645

Acute Oncology Service

CUH · WEEKENDS / BANK HOLIDAYS 9AM–5PM

021 4922000

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00206/

Sources and review status are available online. Follow your own prescription and hospital instructions.